

III The Spatiality of One's Own Body and Motricity

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a. Spatiality of position and spatiality of situation: the body schema.

Let us begin by describing the spatiality of one's own body. If my arm is resting on the table, I will never think to say that it is *next to* the ashtray in the same way that the ashtray is next to the telephone. The contour of my body is a border that ordinary spatial relations do not cross. This is because the body's parts relate to each other in a peculiar way: they are not laid out side by side, but rather envelop each other. My hand, for example, is not a collection of points. In cases of *allochiria*,¹ where the subject senses in his right hand the stimulus that is applied to his left hand, it is impossible to suppose that each of the stimulations individually changes its spatial value,² and the various points on the left hand are transported to the right insofar as they fall within a total organ, within a hand without parts that was displaced all at once. The points, then, form a system, and the space of my hand is not a mosaic of spatial values. Likewise, my entire body is not for me an assemblage of organs juxtaposed in space. I hold my body as an indivisible possession and I know the position of each of my limbs through a *body schema* [*un schéma corporel*]³ that envelops them all. But the notion of the “body schema” is ambiguous, as are all concepts that appear at turning points in science. They can only be fully developed given a reform of methodology. At first they are employed in a sense that is not yet their full sense, and their immanent development is what breaks up previous methods.

“Body schema” was at first understood to be a *summary* of our bodily experience, capable of providing any momentary interoceptivity and proprioceptivity with a commentary and a signification. It was assumed to provide me with the change of position of the parts of my body for each movement of one of them, the position of each local stimulus in the body as a whole, an assessment of the movements accomplished at each moment of a complex gesture, and finally a perpetual translation into visual language of the momentary kinesthetic and articular impressions. By speaking of the body schema, they believed themselves at first simply to be introducing a convenient name designating a large number of image associations, and they merely wanted to express that these were well-established associations constantly ready to come into play. The body schema was thought to develop gradually throughout childhood and to the extent that tactile, kinesthetic, and articular contents associated between themselves or with visual content and were thereby recalled more easily.⁴ As such, it was described physiologically as a center of images in the classical sense. And yet the body schema clearly overflows this associationist definition in the very manner in which psychologists used the concept. For example, in order for the body schema to improve our understanding of *allochiria*, it is not enough that each sensation of the left hand be posited among the generic images of all the parts of the body that would come together to form around the sensation something like a superimposed *sketch* of the body. Rather, these associations must be constantly submitted to a unique law, the spatiality of the body must descend from the whole to the parts, my left hand and its position must be implicated in an overall bodily *plan*

and must have their origin there, such that this hand can suddenly become the right hand, and not merely superimpose itself upon it or fold over it. When one attempts to shed light on the phenomenon of the phantom limb by linking it to the subject's body schema,⁵ then nothing is added to classical explanations involving cerebral traces and renewed sensations unless the body schema, rather than being the residue of habitual cenesthesia,⁶ in fact becomes its law of constitution. If the need was felt to introduce this new word, it was in order to express that the spatial and temporal unity, the inter-sensorial unity, or the sensorimotor unity of the body is, so to speak, an in principle unity, to express that this unity is not limited to contents actually and fortuitously associated in the course of our experience, that it somehow precedes them and in fact makes their association possible.

Thus we are making our way toward a second definition of the body schema: it will no longer be the mere result of associations established in the course of experience, but rather the global awareness of my posture in the inter-sensory world, a “form” in Gestalt psychology's sense of the word.⁷ But the psychologist's analyses in turn overflow this second definition. It is insufficient to say that my body is a form, or a phenomenon in which the whole is anterior to the parts. For how is such a phenomenon possible? Because a form, when compared to the mosaic of the physico-chemical body or to that of the “cenesthesia,” is a new type of existence. If the paralyzed limb of the anosognosic patient no longer counts in the subject's body schema, this is not because the body schema is neither the simple copy, nor even the global awareness of the existing parts of the body; rather, the subject actively integrates the parts according to their value for the organism's projects. Psychologists often say that the body schema is *dynamic*.⁸ Reduced to a precise sense, this term means that my body appears to me as a posture toward a certain task, actual or possible. And in fact my body's spatiality is not, like the spatiality of external objects or of “spatial sensations,” a *positional spatiality*; rather, it is a *situational spatiality*. If I stand in front of my desk and lean on it with both hands, only my hands are accentuated and my whole body trails behind them like a comet's tail. I am not unaware of the location of my shoulders or my waist; rather, this awareness is enveloped in my awareness of my hands and my entire stance is read, so to speak, in how my hands lean upon the desk. If I am standing and if I hold my pipe in a closed hand, the position of my hand is not determined discursively by the angle that it makes with my forearm, my forearm with my arm, my arm with my torso and, finally, my torso with the ground. I have an absolute knowledge of where my pipe is, and *from this* I know where my hand is and where my body is, just as the primitive person in the desert is always immediately oriented without having to recall or calculate the distances traveled and the deviations since his departure. When the word “here” is applied to my body, it does not designate a determinate position in relation to other positions or in relation to external coordinates. It designates the installation of the first coordinates, the anchoring of the active body in an object, and the situation of the body confronted with its tasks. Bodily space can be distinguished from external space and it can envelop its parts rather than laying them out side by side because it is the darkness of the theater required for the clarity of the performance, the foundation of sleep or the vague reserve of power against which the gesture and its goal stand out,⁹ and the zone of non-being *in front of which* precise beings, figures, and points can appear. If my body can ultimately be a “form,” and if there can be, in front of it, privileged figures against indifferent backgrounds, this is insofar as my body is

polarized by its tasks, insofar as it *exists toward* them, insofar as it coils up upon itself in order to reach its goal, and the “body schema” is, in the end, a manner of expressing that my body is in and toward the world.¹⁰ With regard to spatiality, which is our present concern, one's own body is the always implied third term of the figure–background structure, and each figure appears perspectively against the double horizon of external space and bodily space. We must, then, reject as abstract any analysis of bodily space that considers only figures and points, since figures and points can neither be conceived nor exist at all without horizons.

One might reply that the figure–background structure or the point–horizon structure themselves presuppose the notion of objective space, or that, in order to experience a skillful gesture as a figure *on* the solid background of the body, the hand must be united with the rest of the body through this relation of objective space and that, in this way, the figure–background structure again becomes one of the contingent contents of the universal form of space. But what sense could the word “on” have for a subject who could not be situated by his body in front of the world? It implies a distinction between up and down, that is, an “oriented space.”¹¹ When I say that an object is *on* a table, I always place myself (in thought) in the table or the object, and I apply a category to them that in principle fits the relation between my body and external objects. Stripped of this anthropological contribution, the word *on* is no longer distinguished from the word “under” or the term “next to ...” Even if the universal form of space is that without which there would be, for us, no bodily space, it is not that through which there is a bodily space. Even if the form is not the *milieu in which* but rather the *means by which* the content is posited, when it comes to bodily space the form is an insufficient means for this positing, and to this extent the bodily content remains, in relation to it, something opaque, accidental, and unintelligible. The only solution in this direction would be to admit that the body's spatiality has no meaning [*sens*] of its own distinct from objective spatiality, and this would erase the content as a phenomenon and thereby erase the problem of its relation to form.

And yet, can we pretend not to find any distinct sense in the words “on,” “under,” and “next to ...,” or in the dimensions of oriented space? Even if analysis discovers the universal relation of exteriority in all of these relations, the evidentness of up and down, or left and right, for someone who inhabits space prevents us from treating all of these distinctions as mere non-sense and invites us to seek, beneath the explicit sense of the definitions, the latent sense of experiences. The relations between the two spaces would thereby be the following: from the moment I want to thematize bodily space or to work out its sense, I find in it nothing but intelligible space. But at the same time, this intelligible space is not extricated from oriented space, it is in fact nothing but the making explicit of it, and, detached from this source, it has absolutely no sense. Homogeneous space can only express the sense of oriented space because it received this sense from oriented space. If the content can be truly subsumed under the form and can appear as the content *of* this form, this is because the form is only accessible through the content. Bodily space can only truly become a fragment of objective space if, within its singularity as bodily space, it contains the dialectical ferment that will transform it into universal space. This is what we tried to express by saying that the point–horizon structure is the foundation of space. The horizon or the background would not extend beyond the figure or around it if they did not belong to the same genre of being as it, and if they could not be converted into points by a shift of the gaze. But the point–horizon

structure can only teach me what a point is by organizing in advance the zone of corporeality in which the point will be seen and, around this zone, the indeterminate horizons that are the counterpart of this act of seeing. The multiplicity of points or of “heres” can only, in principle, be constituted by an interlocking of experiences in which one of them is perpetually given as an object and that turns itself into the very heart of this space. And, finally, far from my body being for me merely a fragment of space, there would be for me no such thing as space if I did not have a body.

If bodily space and external space form a practical system, the former being the background against which the object can stand out or the void in front of which the object can *appear* as the goal of our action, then it is clearly in action that the spatiality of the body is brought about, and the analysis of movement itself should allow us to understand spatiality better. How the body inhabits space (and time, for that matter) can be seen more clearly by considering the body in motion because movement is not content with passively undergoing space and time, it actively assumes them, it takes them up in their original signification that is effaced in the banality of established situations. Let us examine closely a case of morbid motricity that lays bare the fundamental relations between the body and space.

b. The analysis of motricity in Gelb and Goldstein's study of Schneider.

One patient,¹² whom traditional psychiatry would class among those suffering from psychic blindness, is incapable of performing “abstract” movements with his eyes closed, namely, movements that are not directed at any actual situation, such as moving his arms or legs upon command, or extending and flexing a finger. He cannot describe the position of his body or even of his head, nor the passive movements of his limbs. Finally, when his head, arm, or leg is touched, he cannot say at what point his body was touched; he does not distinguish between two points of contact on his skin, even if they are 80 millimeters apart; he recognizes neither the size nor the form of objects pressed against his body. He only accomplishes abstract movements if he is allowed to see the limb in question, or to execute preparatory movements involving his whole body. The localization of stimuli and the recognition of tactile objects also become possible with the aid of preparatory movements. Even with his eyes closed, the patient executes the movements that are necessary for life with extraordinary speed and confidence, provided they are habitual movements: he takes his handkerchief from his pocket and blows his nose, or takes a match from a matchbox and lights a lamp. He makes wallets by trade, and the output of his work reaches three-quarters the output of a normal worker. He can even,¹³ without any preparatory movements, execute these “concrete” movements on command. In this patient, as well as for patients with cerebellar injuries, a dissociation between the act of pointing and the reactions of taking or grasping can be observed:¹⁴ the same subject who is incapable of pointing to a part of his body on command quickly reaches with his hand for the point at which a mosquito is biting him. We must, then, seek out the reason behind the privilege enjoyed by concrete movements and grasping movements.

c. “Concrete movement.”

Let us take a closer look. One patient asked to point to a part of his body, such as his nose, only succeeds if he is allowed to grasp it. If the patient is directed to interrupt the movement before it reaches its goal,

or if he is only allowed to touch his nose with a wooden ruler, then the movement becomes impossible.¹⁵ It must thus be admitted that “grasping” and “touching” are different from “pointing,” even for the body. From its very beginnings, the grasping movement is magically complete; it only gets under way by anticipating its goal, since the ban on grasping is enough to inhibit the movement. And it must be acknowledged that a point of my body can be present to me as a point to grasp without being presented to me in this anticipated grasp as a point to indicate. But how is this possible? If I know where my nose is when it is a matter of grasping it, how could I not know where my nose is when it is a matter of pointing to it? This is likely the case because the knowledge of a location can be understood in several senses. Classical psychology does not have any concepts for expressing these varieties of the consciousness of location because for it the consciousness of location is always a positional consciousness, a representation, a *Vor-stellung*, because as such it gives us the location as a determination of the objective world and because such a representation either is or is not; but, if it is, then it delivers to us its object without any ambiguity and as an identifiable term throughout all of its appearances. We must, on the contrary, forge here the concepts necessary to express that bodily space can be given to a grasping intention without being given to an epistemic one.

The patient is conscious of bodily space as the envelope of his habitual action, but not as an objective milieu. His body is available as a means of insertion into his familiar surroundings, but not as a means of expression of a spontaneous and free spatial thought. When ordered to perform a concrete movement, he first repeats the order in an interrogative tone of voice, then his body settles into the overall position required by the task, and finally he executes the movement. The whole body can be seen collaborating here, and the patient never reduces it to the strictly indispensable traits as does the normal subject. Along with the military salute come other external marks of respect. Along with the gesture of the right hand that pretends to comb his hair comes the gesture of the left hand that pretends to hold the mirror. Along with the gesture of the right hand that hammers the nail comes the gesture of the left hand that pretends to hold the nail. This is because the instruction is taken literally and because the patient only succeeds in carrying out concrete movements on command on condition of placing himself into the spirit of the actual situation to which they correspond. When the normal subject executes the military salute on command, he sees nothing there but an experimental situation, he thus reduces the movement to its most significant elements and does not fully place himself in the situation.¹⁶ He role-plays with his own body, he amuses himself by playing the soldier, he “irrealizes” himself in the role of the soldier¹⁷ just as the actor slides his real body into the “great phantom”¹⁸ of the character to be performed. The normal subject and the actor do not take the imaginary situations as real, but inversely they each detach their real body from its living situation in order to make it breathe, speak, and, if need be, cry in the imaginary. This is what our patient can no longer do. In life, he says, “I experience movements as a result of the situation, as the sequence of events themselves; my movements and I, we are, so to speak, merely a link in the unfolding of the whole, and I am scarcely aware of any voluntary initiative [...] everything works by itself.” Similarly, in order to execute a movement upon command, he places himself “within the affective situation of the whole, and the movement flows from this whole, just as in life.”¹⁹ If his trick is interrupted and he is reminded of the experimental situation, all of his dexterity disappears. Kinetic

initiation again becomes impossible. The patient must first “find” his arm and “find” the requested gesture through preparatory movements; the gesture itself loses the melodic character that it presents in everyday life and quite clearly becomes a sum of partial movements laboriously placed end to end.

I can thus – by means of my body as a power for a certain number of familiar actions – settle into my surroundings as an ensemble of *manipulanda* without intending my body or my surroundings as objects in the Kantian sense, that is, as systems of qualities linked by some intelligible law, as entities that are transparent, free of all local or temporal adherence, and ready to be named or at least available for a gesture of designation. There is, on the one hand, my arm as the support of these familiar acts, my body as the power of determinate action whose field and scope I know in advance, and my surroundings as the collection of possible points for this power to be applied; there is, on the other hand, my arm as a machine of muscles and bone, as a flexing and extending apparatus, as an articulated object, and the world as a pure spectacle with which I do not merge but that I contemplate and that I point to. As for bodily space, there is clearly a knowledge of location that is reduced to a sort of coexistence with that location but that is not a nothingness, even though it cannot be expressed by a description, nor even by the mute designation of a gesture. The patient bitten by a mosquito need not look for the point of the bite; he finds it immediately, because it is not for him a matter of situating it in relation to axes of coordinates in objective space, but rather of reaching with his phenomenal hand a certain painful place on his phenomenal body. Between the hand as a power for scratching and the point of the bite as a place to be scratched, a lived relation is given in the natural system of one's own body. The operation takes place wholly within the order of the phenomenal, it does not pass through the objective world. Only the spectator, who lends to the subject of movements his own objective representation of the living body, can believe that the bite is perceived, that the hand moves itself in objective space and, consequently, is surprised that the very same subject fails in the designation experiments. Likewise, the subject placed in front of his scissors, his needle, and his familiar tasks has no need to look for his hands or his fingers, for they are not objects to be found in objective space (like bones, muscles, and nerves), but rather powers that are already mobilized by the perception of the scissors or the needle, they are the center-point of the “intentional threads” that link him to the given objects. We never move our objective body, we move our phenomenal body, and we do so without mystery, since it is our body as a power of various regions of the world that already rises up toward the objects to grasp and perceive them.²⁰ Likewise, the patient need not seek a situation and a space in which to deploy concrete movements, this space is itself given, it is the present world: the piece of leather “to be cut” and the lining “to be sewn.” The workbench, the scissors, and the pieces of leather are presented to the subject as poles of action; they define, through their combined value, a particular situation that remains open, that calls for a certain mode of resolution, a certain labor. The body is but one element in the system of the subject and his world, and the task obtains the necessary movements from him through a sort of distant attraction, just as the phenomenal forces at work in my visual field obtain from me, without any calculation, the motor reactions that will establish between those forces the optimum equilibrium, or as the customs of our milieu or the arrangement of our listeners immediately obtains from us the words, attitudes, and tone that fits with them – not that we are trying to disguise our thoughts or simply aiming to please, but because we literally are what others think of us and we are our world. In concrete movement, the patient has neither

a thetic consciousness of the stimulus nor a thetic consciousness of the reaction: quite simply, he is his body and his body is the power for a certain world.

d. Movement toward the possible, “abstract movement.”

What happens, however, in experiments in which the patient fails? If a part of his body is touched and he is asked to locate the point of contact, he begins by putting his entire body into motion and thereby obtains a rough idea of the location, then he makes the location more precise by moving the limb that is being touched and he completes the task by twitching his skin in the area being touched.²¹ If the subject's arm is horizontally extended, then he can only describe its position after a series of pendular movements that present him with the position of the arm in relation to his torso, the position of the forearm in relation to the arm, and the position of his torso in relation to the vertical. In the case of passive movement, the subject senses that there is movement without being able to say what movement and in which direction. Here again he resorts to active movements. The patient deduces that he is lying down from the pressure of the mattress on his back, or that he is standing from the pressure of the ground on his feet.²² If the two points of a compass are placed on his hand, he only distinguishes between them provided he is able to swing his hand and to first bring one point into contact with his skin, then the other. If letters or numbers are traced on his hand, he only identifies them provided he moves his hand himself and it is not the movement of the point on his hand that he perceives, but rather the movement of his hand in relation to the point. This is demonstrated by drawing on his left hand normal letters, which are never recognized, and then the mirror image of the same letters, which are immediately understood. The mere contact of a paper rectangle or oval does not give rise to any recognition, whereas the subject recognizes the shapes if he is allowed exploratory movements that he uses in order to “spell them out,” to locate their “characteristics,” and to deduce the object from them.²³

How are we to make sense of this series of facts, and how should the function that exists for the normal person, but that is missing for the patient, be understood through them? It cannot be a question of simply transferring to the normal person what is missing in the patient and what he is trying to recover. Illness, like childhood or like the “primitive” state, is a complete form of existence, and the procedures that it employs in order to replace the normal functions that have been destroyed are themselves pathological phenomena. The normal cannot be deduced from the pathological, and deficiencies cannot be deduced from their substitutions, through a mere change of sign. The substitutions must be understood as substitutions, as allusions to a fundamental function that they attempt to replace, but of which they do not give us the direct image. The genuine inductive method is not a “method of differences,” it consists in correctly reading phenomena, in grasping their sense, that is in treating them as modalities and variations of the subject's total being. We observe that the patient who is questioned on the position of his limbs or on the location of a tactile stimulus seeks, through preparatory movements, to turn his body into a present object of perception; when questioned about the form of an object touching his body, he seeks to trace it himself by following the contour of the object. Nothing could be more mistaken than to assume that the same operations are at work for the normal person and merely abridged by habit. The patient only seeks these explicit perceptions in order to supply himself with a particular presence of the body and the object that is given for the normal person and that remains for us

to reconstitute. Of course, the perception of the body and of objects in contact with the body is confused for the normal subject as well when there is no movement.²⁴ Nevertheless, the normal subject distinguishes in every case, without movement, between a stimulus applied to his head and a stimulus applied to his body.

Shall we assume²⁵ that the exteroceptive or proprioceptive stimulation has awakened for him “kinesthetic residues” that take the place of actual movements? But how could the tactile givens awaken determinate “kinesthetic residues” if they did not carry with them some characteristic that makes them capable of this, if they did not themselves have a precise or a confused spatial signification?²⁶ We shall say then, at the very least, that the normal subject immediately has several “holds”²⁷ on his body. He does not have his body available merely as implicated in a concrete milieu, he is not merely situated in relation to the tasks set by his trade, nor is he merely open to real situations. Rather, in addition he possesses his body as the correlate of pure stimuli stripped of all practical signification; he is open to verbal and fictional situations that he can choose for himself or that a researcher might suggest. His body is not presented to him through touch as a geometrical plan upon which each stimulus would come to occupy an explicit position, and Schneider's illness consists in precisely the need to convert the touched part of his body into a figure in order to know where he is being touched. But each bodily stimulation for the normal subject awakens, not an actual movement, but a sort of “virtual movement”; the part of the body addressed escapes from anonymity, appears through a strange tension, and as a certain power for action within the frame of the anatomical apparatus. The normal subject's body is not merely ready to be mobilized by real situations that draw it toward themselves, it can also turn away from the world, apply its activity to the stimuli that are inscribed upon its sensory surfaces, lend itself to experiments and, more generally, be situated in the virtual. Because the patient is enclosed in the actual, the pathological sense of touch needs its own movements in order to localize the stimuli, and again it is for the same reason that the patient substitutes for recognition and tactile perception the laborious decoding of stimuli and the deduction from objects. For example, for a key to appear in my tactile experience as a key, touch must have a sort of fullness, a tactile field where isolated impressions can be integrated into a configuration, just as notes are no longer the points of passage of a melody; and the same viscosity of tactile givens that subjects the body to actual situations also reduces the object to a sum of successive “characteristics,” reduces perception to an abstract signaling, reduces recognition to a rational synthesis or a probable conjecture, and strips the object of its carnal presence and its facticity. Whereas for the normal person each motor or tactile event gives rise in consciousness to an abundance of intentions that run from the body as a center of virtual action either toward the body itself or toward the object, for the patient, on the contrary, the tactile impression remains opaque and closed in upon itself. It can surely draw the grasping hand toward itself, but it is not laid out before that hand as something that could be pointed to. The normal person *reckons with* the possible, which thus acquires a sort of actuality without leaving behind its place as a possibility; for the patient, however, the field of the actual is limited to what is encountered in real contact or linked to these givens through an explicit deduction.

e. Motor project and motor intentionality.

For these patients, the analysis of “abstract movement” shows even more clearly this possession of space, or this spatial existence that is the primordial condition of every living perception. If the patient is asked to execute an abstract movement with his eyes closed, a series of preparatory operations are necessary for him “to find” the operative limb itself, the direction or the pace of the movement, and finally the level on which it will unfold. If, for example, he is simply asked to move his arm, he is at first dumbfounded. Then he moves his whole body and the movements are subsequently restricted to the arm that, in the end, the subject “finds.” If it is a question of “raising his arm,” the patient must also “find” his head (which is for him the symbol of “up”) by a series of pendular oscillations that are carried out throughout the duration of the movement and that establish the goal. If he is asked to trace a square or a circle in the air, he first “finds” his arm, then he brings his hand forward, just as a normal subject does in order to locate a wall in the dark, and finally he attempts several movements in a straight line and along various curves, and if one of these movements happens to be circular, he promptly completes the task. Moreover, he only succeeds in finding the movement on a certain plane that is just about perpendicular with the ground, and, outside of this privileged plane, he does not even know how to begin.²⁸

Clearly the patient's body is only available to him as an amorphous mass in which only real movements introduce divisions and articulations. He leaves the care of executing the movement to his body, like an orator who could not speak a word without relying upon a text written in advance. The patient does not seek and does not find the movement himself; rather, he agitates his body until the movement appears. The instructions he receives are not meaningless for him, since he knows how to recognize what remains imperfect in his first attempts, and since, if the accidents of the gesticulating bring about the requested movement, he also knows to recognize and to make use of this good fortune. But even if the instructions have for him an *intellectual signification*, they do not have a *motor signification*, they do not speak to him as a motor subject. He can surely discover in the trace of a completed movement the illustration of the given instructions, but he can never deploy the thought of a movement into an actual movement. He is missing neither Motricity nor thought, and we must acknowledge, between movement as a third person process and thought as a representation of movement, an anticipation or a grasp of the result assured by the body itself as a motor power, a “motor project” (*Bewegungsentwurf*), or a “motor intentional-ity” without which the instructions would remain empty. Sometimes the patient thinks of the ideal formula of the movement, other times he throws his body into blind attempts; however, for the normal person every movement is indissolubly movement and consciousness of movement. This can be expressed by saying that, for the normal person, every movement has a *background*, and that the movement and its background are “moments of a single whole.”²⁹ The background of the movement is not a representation associated or linked externally to the movement itself; it is immanent in the movement, it animates it and guides it along at each moment. For the subject, the beginning of kinetic movement is, like perception, an original manner of relating to an object.

The distinction between abstract movement and concrete movement is thereby clarified: the background of concrete movement is the given world, the background of abstract movement is, on the

contrary, constructed. When I motion to my friend to approach, my intention is not a thought that I could have produced within myself in advance, nor do I perceive the signal in my body. I signal across the world; I signal over there, where my friend is. The distance that separates us and his consent or refusal are immediately read in my gesture. There is not first a perception followed by a movement, the perception and the movement form a system that is modified as a whole. If, for example, I realize that my friend does not want to obey, and if I thereby modify my gesture, we do not have here two distinct conscious acts. Rather, I see my partner's resistance, and my impatient gesture emerges from this situation, without any interposed thought.³⁰ If I now execute the “same” movement, but without aiming at a present or even an imaginary partner, that is, the same movement as “a sequence of movements in themselves,”³¹ if in other words I execute a “bending” of the forearm toward the arm with a “supination” of the arm and a “bending” of the fingers, then my body, which was just previously the vehicle of movement, now becomes the goal of movement. Its motor project no longer aims at someone in the world: it aims at my forearm, my arm, my fingers, and it aims at them insofar as they are capable of breaking with their insertion in the given world and of sketching out around me a fictional situation, or when I do not even have a fictional partner, insofar as I curiously examine this strange signifying machine and set it to work for my own amusement.³²

f. The “function of projection.”*

Within the busy world in which concrete movement unfolds, abstract movement hollows out a zone of reflection and of subjectivity, it superimposes a virtual or human space over physical space. Concrete movement is thus centripetal, whereas abstract movement is centrifugal; the first takes place within being or within the actual, the second takes place within the possible or within non-being; the first adheres to a given background, the second itself sets up its own background. The normal function that makes abstract movement possible is a function of “projection” by which the subject of movement organizes before himself a free space in which things that do not exist naturally can take on a semblance of existence. There are patients, not as severely affected as Schneider, who perceive forms, distances, and objects themselves, but who can neither trace out the directions useful for action upon these objects, nor arrange them according to a given principle, nor in general apply the anthropological determinations to the spatial spectacle that would turn it into the landscape of our action. When these patients are, for example, placed in a labyrinth and confronted by an impasse, they only discover the “opposite direction” with difficulty. If a ruler is placed between them and the doctor, they do not know how to arrange objects to be “on their side” or “on the doctor's side.” They are inaccurate when indicating on another person's arm a point that is being stimulated on their own body. Knowing that today is a Monday in March, they will have difficulty indicating the previous day and month, even though they know the sequence of days and months by heart. They do not succeed in comparing the number of units contained in two sets of sticks placed in front of them: sometimes they count the same stick twice, sometimes they count sticks from one set with those that belong to the other.³³ This is because all of these operations require the same power of marking out borders and directions in the given world, of establishing lines of force, of arranging perspectives, of organizing the given world according to the projects of the moment,

and of constructing upon the geographical surroundings a milieu of behavior and a system of significations that express, on the outside, the internal activity of the subject. The world no longer exists for these patients except as a ready-made or fixed world, whereas the normal person's projects polarize the world, causing a thousand signs to appear there, as if by magic, that guide action, as signs in a museum guide the visitor. This function of "projection" or "conjuring up" [*évation*] (in the sense in which the medium conjures up and makes the dead person appear) is also what makes abstract movement possible. For, in order to possess my body independently of all urgent tasks, in order to make use of it in my imagination, and in order to trace in the air a movement that is only defined by a verbal instruction or by moral necessities, I must also invert the natural relation between my body and the surroundings, and a human productivity must appear through the thickness of being.

g. i These phenomena are impossible to understand through a causal explanation and by connecting them to a visual deficiency ...

These are the terms in which the movement disorders we are considering can be described. But one might find that this description (as has often been said of psychoanalysis)³⁴ only reveals the sense or the essence of the illness and does not give us its cause. Science would only begin with an explanation that must seek beneath phenomena the conditions on which they depend according to the proven methods of induction. Here, for example, we know that Schneider's motor disorders coincide with extensive disorders of the visual function, themselves linked to the occipital injury at the origin of the condition. Through vision alone, Schneider does not recognize any object.³⁵ His visual givens are nearly formless patches.³⁶ As for absent objects, he is incapable of forming a visual representation of them.³⁷ On the other hand, we know that "abstract" movements become possible for the subject the moment he focuses his eyes upon the limb charged with the task.³⁸ Thus, what remains of voluntary motricity depends upon what remains of visual knowledge. Mill's famous methods would allow us to conclude here that abstract movements and *Zeigen* [pointing] depend upon the power of visual representation, and that the concrete movements retained by the patient, along with imitative movements by which he compensates for the poverty of his visual givens, come from the kinesthetic or tactile sense, which are, in fact, remarkably exploited by Schneider. The distinction between concrete and abstract movement, like that between *Greifen* [grasping] and *Zeigen*, would allow itself to be reduced to the classical distinction between the tactile and the visual, and the function of projection or conjuring up, which we brought to light just above, would be reduced to perception and visual representation.³⁹

In fact, an inductive analysis carried out according to Mill's methods does not reach any conclusion. For the disorders of abstract movement or of *Zeigen* are not only encountered in the case of psychic blindness, but also in cerebellar injuries and in many other diseases.⁴⁰ Among all of these concordances, it is not permitted to choose one of them alone as the decisive one and to "explain" the act of pointing through it. Faced by the ambiguity of the facts, one can only renounce the simple statistical recording of coincidences and seek to "understand" the relation manifested by them. In the case of cerebellar injuries, it is observed that visual stimuli, in contrast to sonorous stimuli, only give rise to imperfect motor

reactions, and nevertheless there is no reason in these cases to assume a primary disturbance of the visual function. The designating movements do not become impossible because the visual function is affected; it is, on the contrary, because the *Zeigen* attitude is impossible that the visual stimuli only give rise to imperfect reactions. We should acknowledge that sound, of itself, calls forth rather a grasping movement, while visual perception calls forth a designating gesture. "Sound always directs us toward its content, its signification for us; in visual presentation, to the contrary, we can much more easily 'abstract' from the content and we are much more oriented toward the location in space where the object is situated."⁴¹ A sense is thus defined less by the indescribable quality of its "psychic contents," than by a certain way of offering its object, that is, by its epistemological structure of which the quality is but the concrete realization and, to use Kant's term, the exhibition. The doctor who applies "visual" or "sonorous stimuli" to the patient believes he is testing the patient's "visual" or "auditive sensitivity" and thus taking inventory of the sensible qualities that compose his consciousness (in empiricist language) or the material available to his knowledge (in intellectualist language). The doctor and the psychologist borrow the concepts of "vision" and "hearing" from common sense, and common sense takes them to be univocal because our body indeed comprises distinct visual and auditory mechanisms to which common sense – according to a general postulate of "constancy"⁴² that expresses our natural ignorance of ourselves – assumes isolatable conscious contents must correspond. But when taken up and applied systematically by science, these confused concepts hinder research and ultimately reveal the need for a general revision of these naïve categories. In fact, what the threshold measurement actually tests for are functions anterior to the specification of sensible qualities and anterior to the deployment of knowledge; it tests the manner in which the subject makes that which surrounds him exist for him: either as the pole of activity and the term of an act of grasping or releasing, or as a spectacle and the theme of knowledge. The motor disorders in cerebellar injury cases and those of psychic blindness can only be coordinated if the background of movement and vision is defined not by a stock of sensible qualities, but by a certain manner of articulating or of structuring the surroundings. We are thus led back by the very use of inductive method to these "metaphysical" questions that positivism wanted to avoid. Induction only reaches its end if it is not restricted to recording presences, absences, and concomitant variations, and if it conceives of and understands facts as subsumed under ideas not contained in them. There is no choice between a description of the illness that would give us its sense and an explanation that would give us its cause, and there are no explanations without understanding.

But let us hone our objection, which upon analysis splits into two. (1) The "cause" of a "psychic fact" is never another "psychic fact" that could be discovered by simple observation. For example, visual representation does not explain abstract movement because this representation is itself inhabited by the same power of projecting a spectacle that appears in abstract movement and in the designating gesture. But this power does not appear to the senses, and not even to inner sense. Let us say provisionally that it is only revealed to a certain type of reflection whose nature we will make more precise below. From this, it follows directly that psychological induction is not a simple inventory of facts. Psychology does not explain by designating, among facts, the constant and unconditioned antecedent. It conceives or understands facts, just as induction in physics is not restricted to recording empirical sequences and rather creates concepts that are capable of coordinating facts. This is why no induction – in psychology

or in physics – can boast a critical experiment. Since the explanation is invented, not discovered, it is never given with the fact, it is always a probable interpretation. So far we have merely applied to psychology what has been fully established with regard to induction in physics,⁴³ and our first objection weighs against the empiricist manner of conceiving induction and against Mill's methods. – (2) Now, we will see that this first objection overlaps with a second. In psychology, it is not only empiricism that needs to be rejected, but rather the inductive method and causal thought in general. The object of psychology is of such a nature that it could not be determined by relations of function to variable. Let us establish these two points in some detail.

(1) We observe that Schneider's motor disorders are accompanied by an extensive deficiency in visual knowledge. We are thus tempted to consider psychic blindness as a differential case of pure tactile behavior and, since the consciousness of bodily space and of abstract movement (which aims at virtual space) are almost completely missing for him, we are inclined to conclude that touch in itself gives us no experience of objective space.⁴⁴ We will say, then, that touch is not by itself apt to offer a background to movement, that is, to lay out before the moving subject his point of departure and his point of arrival in a strict simultaneity. The patient attempts to provide a “kinesthetic background” for himself through preparatory movements, and he succeeds in this way at “noting” the position of his body at the outset and in beginning the movement. Nevertheless, this kinesthetic background is labile; it cannot, like a visual background, reveal to us the trajectory of the moving object in relation to its point of departure and point of arrival during the entire duration of its movement. This kinesthetic background is disrupted by the movement itself, and it needs to be reconstructed after each phase of movement. This is why, we will say, Schneider's abstract movements have lost their melodic character, why they are built out of fragments placed end to end, and why they often “derail” along the way. Schneider's missing practical field is nothing other than the visual field.⁴⁵

But in order to be justified in linking motor disorders and visual disorders in psychic blindness, or the function of projection to vision as its constant and unconditioned antecedent for the normal subject, we would have to be certain that only the visual givens were affected by the illness, and that all other behavioral conditions (and particularly tactile experience) remained what they were for the normal subject. Can this be confirmed? Here is where we will see that the facts are ambiguous, that no critical experiment is possible, and that no explanation is final. If we observe that a normal subject is capable of executing abstract movements with his eyes closed, and that his tactile experience is sufficient to govern motricity, then one might always respond that the tactile givens of the normal subject have in fact received their objective structure from the visual givens, according to the previous schema of training of the senses. If we observe that a blind man is capable of localizing the *stimuli* on his body and of executing abstract movements (notwithstanding the fact that there are examples of preparatory movements among the blind), one might still respond that the frequency of associations has communicated the qualitative coloration of kinesthetic impressions to the tactile impressions, and has welded them into a quasi-simultaneity.⁴⁶ And yet, several facts leave the impression of a primary alteration of tactile experience even in the behavior of the patients in question.⁴⁷ For example, one subject who knows how to knock on a door can no longer perform the action if the door is hidden or if it

is simply out of reach. In this latter case, the patient cannot execute the gesture of knocking on the door, or of opening it, *even if his eyes are open and focused on the door*.⁴⁸ How could visual deficiencies be blamed here, when the patient has a visual perception of the goal available that ordinarily suffices to more or less orient his movements? Have we not revealed a primary disturbance of touch? Clearly, in order for an object to trigger a movement it must be included in the motor field of the patient, and the disorder consists in a shrinking of the motor field, from now on limited to the actually tangible objects and to the exclusion of that horizon of possible touching that surrounds them for the normal subject. This deficiency would ultimately be related to a function deeper than vision, and also deeper than touch (as a sum of given qualities); it would concern the subject's living region, that opening up to the world that ensures that objects currently out of reach nevertheless count for the normal subject, that they exist as tactile for him and remain part of his motor universe. According to this hypothesis, when patients observe their hand and the goal throughout the entire duration of the movement,⁴⁹ we must not see this as the mere magnification of a normal procedure, and in fact this recourse to vision could only be made necessary through the collapse of virtual touching. If, however, we remain on a strictly inductive level, this interpretation, which puts touch into question, remains optional; one can still prefer (with Goldstein) another interpretation: in order to knock, the patient needs a goal within reach, precisely because vision, which is deficient for him, no longer suffices to provide a solid background for the movement. Thus, there is no fact that could establish, in a decisive manner, that the tactile experience of these patients is or is not identical to that of normal subjects, and Goldstein's conception, like the physical theory, can always be harmonized with the facts by means of some auxiliary hypothesis. Just as in physics, no strictly exclusive interpretation is possible in psychology.

Nevertheless, if we look more closely we will see that the impossibility of a critical experiment is grounded in psychology on peculiar reasons: it comes from the very nature of the object under investigation – behavior – and this impossibility has much more decisive consequences. Given a set of theories in which none are absolutely excluded and of which none are rigorously established by the facts, physics can all the same choose according to the degree of plausibility, that is, according to the number of facts that each one succeeds in coordinating without weighing itself down with auxiliary hypotheses believed to respond to the needs of the case. In psychology, this criterion leads us into error: we have just seen that no auxiliary hypothesis is necessary to explain the impossibility of the “knocking” gesture in front of the door through the visual disorder. Not only do we never reach an exclusive interpretation (a deficiency of virtual touching *or* a deficiency of the visual world), but moreover we must necessarily deal with *equally probable* interpretations, because “visual representations,” “abstract movement,” and “virtual touching” are only different names for a single central phenomenon. Psychology, then, does not find itself in the same situation as physics, that is, confined to the probability of inductions; it is incapable of choosing, even according to plausibility, between hypotheses that nevertheless remain incompatible from the strictly inductive point of view. For an induction, even a merely probable one, to remain possible, the “visual representation” or the “tactile perception” must cause the abstract movement, or they must in the end both be effects of another cause. The three or four terms must be able to be considered from the outside, and one must be able to discover correlative variations in them. But, if they were not separable, if each of them presupposed the others, then the

failure would not be the failure of empiricism or of the attempts to find critical experiments, it would be the failure of inductive method or of causal thought in psychology. Thus, we arrive at the second point we wanted to establish.

(2) If, as Goldstein acknowledges, the coexistence of the tactile givens with the visual givens modifies the normal subject's tactile givens profoundly enough for them to be able to serve as the background of abstract movement, then the tactile givens of the patient – cut off from this visual support – will not be able to be directly identified with those of the normal subject. Tactile and visual givens, says Goldstein, are not juxtaposed for the normal subject, the former owe to the nearness of the latter a “qualitative nuance” that they have lost for Schneider. This is to say, he adds, that the study of the purely tactile is impossible in the normal person, and that the disorder alone provides a picture of what tactile experience, reduced to itself, might look like.⁵⁰ This conclusion is correct, but it amounts to saying that the word “touching” does not have the same sense when applied to the normal subject and to the patient, that the “purely tactile” is a pathological phenomenon that does not enter as a component of normal experience, that the disorder, by disorganizing the visual function, had not revealed the pure essence of the tactile, that it had rather modified the entire experience of the subject. Or again, if one prefers, it is to say that there is, for the normal subject, not a tactile experience and a visual experience, but rather an integral experience where it is impossible to measure out the different sensory contributions. Experiences mediated by touch in psychological blindness have nothing in common with those mediated by touch for the normal subject, and neither are worthy of being called “tactile” givens. Tactile experience is not an isolated condition that could be held constant while “visual” experience is varied in such a way as to locate the causality belonging to each, nor is behavior a function of these variables, it is presupposed in their definition, just as each one is presupposed in the definition of the other.⁵¹ Psychic blindness, deficiencies in the sense of touch, and motor disorders are three *expressions* of a more fundamental disturbance by which they can be understood, not three components of morbid behavior. Visual representations, tactile givens, and motricity are three phenomena cut out of the unity of behavior. If, because they present correlated variations, we want to explain one in terms of the others, then we forget, for example, that the act of visual representation, as is shown by the case of cerebellar injury, already presupposes the very power of projection that also appears in abstract movement and in the designating gesture, and so we assume what was meant to be explained. By enclosing within vision, touch, or some actual given the power of projection that actually inhabits them all, inductive and causal thought conceals this power from us and makes us blind to the dimension of behavior that is precisely that of psychology.

In physics, of course, the establishing of a law demands that the scientist conceive of the idea under which the facts will be coordinated and this idea, which is not found in the facts, will never be verified through a critical experiment, it will only ever be probable. But it is still the idea of a causal link in the sense of a relation between a function and a variable. Atmospheric pressure had to be invented, but, after all, it was still a third person process, a function of a number of variables. If behavior is a form in which “visual contents” and “tactile contents,” or sensitivity and motricity, only figure as inseparable moments, then it remains inaccessible to causal thought. Behavior can only be grasped by another type of thought – the type of thought that takes its object in its nascent state, such as it appears to him who lives it, with

the atmosphere of sense by which it is enveloped, and that seeks to slip itself into this atmosphere in order to discover, behind dispersed facts and symptoms, the total being of the subject in the case of the normal person or the fundamental disorder in the case of the patient.

g. ii ... Or through a reflective analysis and by connecting them to the "symbolic function."]

If we can explain neither the disorders of abstract movement through the loss of visual contents, nor consequently the function of projection through the actual presence of these contents, then only one method seems possible: it would consist in reconstituting the fundamental disorder by following the symptoms backward, not to an observable *cause*, but to an intelligible *reason* or condition of possibility; that is, to treat the human subject as an indivisible consciousness that is wholly present in each of its manifestations. If the disorder should not be related to content, then it must be linked to the form of knowledge; if psychology is not empiricist and explanatory, then it must be intellectualist and reflective.

Precisely like the act of naming,⁵² the act of pointing presupposes that the object, rather than being approached, grasped, and engulfed by the body, be maintained at a distance and sketch out an image in front of the patient. Plato still allowed the empiricist the power of definition by pointing, but in fact even this silent gesture is impossible if *what* it designates is not already ripped out of instantaneous and monadic existence and treated as the representative of its previous appearances in me and of its simultaneous appearances in others; that is, unless it is subsumed under a category and promoted to the status of an idea. If the patient can no longer point to a place on his body that is being touched, this is because he is no longer a subject facing an objective world and because he can no longer take up the "categorical attitude."⁵³ Abstract movement is likewise compromised insofar as it presupposes a consciousness of the goal, insofar as it is carried by it and is a movement for itself. And indeed, no existing object can trigger abstract movement, it is clearly centrifugal, it sketches out in space an unmotivated intention that falls upon one's own body and constitutes it as an object rather than moving through it in order to meet up with things by its means. Abstract movement is thus inhabited by a power of objectification, by a "symbolic function,"⁵⁴ a "representation function,"⁵⁵ or a power of "projection"⁵⁶ that, moreover, is already at work in the constitution of "things" and that consists in treating the sensory givens as representatives of each other and, when taken together, as representatives of an "*eidos*." It consists in giving them a sense, in animating them from within, in organizing them into a system, in centering a plurality of experiences upon a single intelligible core, and in making an identifiable unity appear in them under different perspectives. In short, it consists in arranging behind the flux of impressions an invariant that gives the flux its reason and in articulating the material of experience.

But it cannot be said that consciousness *has* this power; rather, it *is* this power itself. From the moment there is consciousness, and in order for consciousness to exist, there must be something of which it is conscious, an intentional object, and it can only bear upon this object insofar as it "irrealizes" itself and throws itself into the object, insofar as it is entirely within this reference to ... something, and insofar as it is a pure act of signification. If a being is consciousness, it must be nothing other than a fabric of intentions. If he ceases to be defined by the act of signifying, then he falls back to the status of

a thing, the thing being precisely that which does not know, that which remains within an absolute ignorance of itself and of the world, that which, consequently, is not a genuine “self,” that is, a “for itself,” and that has merely a spatio-temporal individuation and existence in itself.⁵⁷ Consciousness will thus not admit of degrees. If the patient no longer exists as a consciousness, then he must exist as a thing. Either movement is movement for itself, in which case the “stimulus” is not the cause but rather the intentional object, or movement breaks apart and scatters throughout existence in itself, it becomes an objective process in the body whose phases are successive but unknown to each other. The privilege of concrete movements in the disorder would be explained because they are reflexes in the classical sense. The patient's hand finds the mosquito's location on his body because the preestablished nervous circuits direct the reaction to the place of stimulation. The movements involved in his regular work are preserved because they depend upon firmly established conditioned reflexes. They subsist despite the psychic deficiencies because they are movements in themselves. The distinction between concrete and abstract movement, between *Greifen* and *Zeigen*, would be the distinction between physiology and psychology, between existence in itself and existence for itself.⁵⁸

We will see that the first distinction, far from mapping onto the second, is in fact incompatible with it. Every “physiological explanation” tends to be generalized. If the grasping movement or concrete movement is guaranteed by a connection between each point on the skin and the motor muscles that guide the hand to it, then it is not clear why the same nervous circuit directing the same muscles in a movement that is scarcely different could not guarantee the *Zeigen* gesture just as well as the *Greifen* movement. The physical difference between the mosquito stinging the skin and the wooden ruler that the doctor presses against the same location does not suffice to explain that the grasping movement is possible while the designating gesture is impossible. The two “stimuli” are only genuinely distinguished if we take into consideration their affective value or their biological sense; the two responses only cease to merge if *Zeigen* and *Greifen* are considered as two different ways of relating to the object and two types of being in the world. But this distinction is precisely what is impossible once the living body has been reduced to the condition of an object. If one concedes even once that the living body is the seat of a third person process, then nothing in behavior can be reserved for consciousness. Gestures and movements, since they make use of the same organ-objects and the same nerve-objects, must be laid out upon the stage of surface-level processes and must be inserted into the closely woven fabric of “physiological conditions.” When the patient, engaging in his work, lifts his hand toward a tool that is on the table, does he not shift the segments of his arm exactly as he would have to in order to execute an abstract movement of extension? Does not an everyday gesture contain a series of muscular contractions and of innervations? The physiological explanation, then, cannot be limited.

On the other hand, it is also impossible to limit consciousness. If the gesture of pointing is related to consciousness, and if the stimulus can even once cease to be the cause of the reaction in order to become the intentional object, then it is inconceivable that it could, in any case, function as a pure cause or that movement could ever be blind. For if “abstract” movements, in which there is a consciousness of the point of departure and the point of arrival, are to be possible, then we must know at each moment of our lives just where our body is without having to go looking for it (in the manner that we go looking for an object that was moved during our absence). Even “automatic” movements must be anticipated in

consciousness – that is, in our body there are never any movements in themselves. And if every objective space exists only for intellectual consciousness, then we must discover the categorial attitude even in the grasping movement.⁵⁹ Like physiological causality, the appearance of consciousness [*prise de conscience*] cannot begin at some particular point. Either the physiological explanation must be rejected, or it must be acknowledged as total; either consciousness must be denied, or it must be acknowledged as total. We cannot relate certain movements to the bodily mechanism and certain other ones to consciousness, the body and consciousness are not mutually limiting, they can only be parallel. Every physiological explanation generalizes into a mechanistic physiology, every appearance of consciousness generalizes into an intellectualist psychology, and mechanistic physiology or intellectualist psychology level out behavior and efface the distinction between abstract movement and concrete movement, between *Zeigen* and *Greifen*. The distinction can only be maintained if there are *several ways for the body to be a body, and several ways for consciousness to be consciousness*. So long as the body is defined through existence in itself, it functions uniformly as a mechanism; so long as the soul is defined by pure existence for itself, it will only know objects laid out in front of it. The distinction between abstract and concrete movement does not, then, merge with the distinction between the body and consciousness, it does not belong to the same reflective dimension, it only finds its place in the dimension of behavior. Pathological phenomena make something flicker before our eyes, but this something is not the pure consciousness of the object. Like the diagnosis offered by empiricist psychology, intellectualist psychology's diagnosis – that we are seeing a collapse of consciousness and a setting free of automatic reflexes – would miss the fundamental disorder.

h. The existential ground of the “symbolic function” and the structure of illness.

The intellectualist analysis, here as everywhere, is less false than it is abstract. The “symbolic function” or “representation function” certainly underlies our movement, but it is not an ultimate term for the analysis, it in turn rests upon a certain ground. Intellectualism's error is to make it depend upon itself, to separate it from the materials in which it is realized, and to recognize in us, as originary, a direct presence in the world. For beginning from this transparent consciousness, and from this intentionality that does not admit of degrees, everything that separates us from the true world – error, illness, madness, and, in short, embodiment – is reduced to the status of a mere appearance. Of course, Intellectualism does not set up consciousness apart from its materials, and, for example, it explicitly resists introducing a “symbolic consciousness” behind speech, action, and perception that would be the common and numerically single form of linguistic, perceptual, and motor materials. There is no “general symbolic faculty,”⁶⁰ says Cassirer, and reflective analysis does not look to establish a “community of being,” but rather a “community of sense”⁶¹ among the pathological phenomena that concern perception, language, and action. Intellectualist psychology, given that it has definitively left causal thought and realism behind, would be capable of seeing the sense or the essence of the disorder and of recognizing a unity of consciousness that is not observable on the level of being and that is self-affirmed upon the level of truth. But precisely this distinction between the community of being and the community of sense, the conscious passage from the order of existence to the order of truth, and the reversal that allows for the

affirmation of sense and value as autonomous are, in practice, equal to an abstraction, since, from the point of view ultimately adopted, the variety of phenomena becomes meaningless and incomprehensible. If consciousness is placed outside of being, then consciousness can never be penetrated by being. The empirical variety of consciousnesses – morbid consciousness, primitive consciousness, infantile consciousness, the consciousness of others – cannot be taken seriously; there is nothing there to be known or comprehended. One thing alone is comprehensible, namely, the pure essence of consciousness. None of these other consciousnesses could fail to actualize the *Cogito*. *Behind* his delusions, obsessions, and lies, the madman *knows that he is* delirious, that he makes himself obsessive, that he lies, and ultimately that he *is not* mad, *he just thinks he is*. On this account, then, everything is just fine and madness is simply a lack of good will. The analysis of the sense of the disorder, once it posits the symbolic function, identifies all disorders, reduces aphasia, apraxia, and agnosia to a unity,⁶² and perhaps even has no means of distinguishing them from schizophrenia.⁶³

We can thus understand why doctors and psychologists decline Intellectualism's invitation, and go back, lacking a better explanation, to attempts at causal explanation that at least have the advantage of bringing into the account what is peculiar to the disorder and to each particular disorder, and which thereby offer us at least the illusion of actual knowledge. Modern pathology shows that there is no strictly elective disorder, but it also shows that each disturbance is nuanced according to the region of behavior that it principally attacks.⁶⁴ Even if every case of aphasia, when closely observed, included gnostic⁶⁵ and practical disturbances, even if every case of apraxia included disturbances of language and perception, and even if every case of agnosia included disturbances of language and action, it remains the case that the center of these disturbances is here in the zone of language, there in the zone of perception, and lastly in the zone of action. When the symbolic function is blamed in each of these cases, the common structure of different disturbances is certainly characterized, but this structure must not be detached from the materials where it is, if not electively, then principally produced each time. After all, Schneider's disorder is not initially metaphysical, for it was a piece of shrapnel that injured him in his occipital region. His visual deficiencies are extensive, but as we have said it would be absurd to explain all others through them as if through their cause, and it is no less absurd to think that the shrapnel collided with symbolic consciousness. Rather, his Spirit is affected through vision.

So long as the means of linking the origin and the essence of the disorder has not been found, so long as a *concrete essence* or a *structure* of the illness that expresses both its generality and its particularity has not been found, so long as phenomenology has not become genetic phenomenology, then these offending retreats into causal thought and naturalism will remain justified. Our question thus comes into focus. We must imagine, between the linguistic, perceptual, and motor contents and the form that they receive or the symbolic function that animates them, a relationship that would be neither the reduction of the form to the content, nor the subsumption of the content under an autonomous form. We must understand how Schneider's disorder simultaneously overflows the particular contents of his experience (visual, tactile, and motor) and yet only attacks the symbolic function through the privileged material of vision. The senses, and one's own body overall, present the mystery of a whole that, without leaving behind its *haecceity* and its particularity, emits beyond itself significations capable of offering a

framework for an entire series of thoughts and experiences. If Schneider's disorder has to do with motricity and thought as much as with perception, it remains the case that for thought the disorder primarily affects his power of recognizing simultaneous wholes, and for motricity it primarily affects his power of surveying movement from above [*survoler*] and of projecting it into the exterior world. Thus, mental space and practical space are, in a way, destroyed or damaged, and the words themselves are a sufficient indication of the genealogy of the disorder in vision. The visual disorder is not the cause of other disorders, nor of the particular disorder affecting thought. But neither is it a simple consequence of them. Visual contents are not the cause of the function of projection, but no more is vision a simple opportunity for the Spirit to deploy a power that is in itself unconditioned. Visual contents are taken up, utilized, and sublimated to the level of thought through a symbolic power that transcends them, but this power can only be constituted on the basis of vision. Phenomenology calls the relation between matter and form a relation of *Fundierung* [founding]: the symbolic function does not depend on vision as its ground because vision is its cause, but because vision is this gift of nature that Spirit had to make use of beyond all expectations, to which it had to give a radically new sense and upon which nevertheless it depended, not merely in order to become embodied, but even in order to exist at all. Form absorbs content to the point that content ultimately appears as a mere mode of form, and the historical preparations of thought appear as a ruse of Reason disguised as Nature. But reciprocally, even in its intellectual sublimation, content remains radically contingent as the initial institution or founding⁶⁶ of knowledge and action, as the first grasp of being or of value whose concrete richness will never be exhausted by knowledge or action, and whose spontaneous method they will everywhere renew. We must restore this dialectic between form and matter, or rather, since “reciprocal action” is nothing more than a compromise with causal thought and a contradictory formulation, we must describe the milieu where this contradiction is conceivable; in other words, existence, or the perpetual taking up of fact and chance by a reason that neither exists in advance of this taking up, nor without it.⁶⁷

If we wish to see what underlies the “symbolic function” itself, we must first understand that even intelligence does not fit well with intellectualism. What compromises thought for Schneider is not that he is incapable of perceiving the concrete givens as exemplars of a unique *eidos*, or of subsuming them under a category; rather, it is that he can only link them through an explicit subsumption. We observe, for example, that the patient does not understand analogies as simple as: “fur is to the cat what feathers are to the bird,” or “light is to a lamp what heat is to a stove,” or again, “the eye is to light and color what the ear is to sounds.” He similarly fails to understand terms in their usual metaphorical sense, such as “the foot of the chair” or “the head of the nail,” even though he knows what part of the object these words designate. Sometimes normal subjects with the same education do not know how *to explain* the analogy any better than the patient, but this is for opposite reasons. It is easier for the normal subject to understand the analogy than it is for him to analyze it, whereas the patient only succeeds in understanding it when he has made it explicit through a conceptual analysis. “He seeks [...] a common material property from which he could deduce the identity of the two relations as if from a middle term.”

⁶⁸ For example, he reflects upon the analogy between the eye and the ear and clearly only understands it when he can say: “the eye and the ear are both sense organs, thus, they must produce something

similar.” If we were to describe the analogy as the apperception of two given terms under one concept that coordinates them, then we would be giving as normal a procedure that is nothing other than pathological and that represents the detour through which the patient must go in order to offer a substitution for a normal understanding of the analogy.

This freedom in the choice of a *tertium comparationis*⁶⁹ is, for the patient, the opposite of the intuitive determination of the image for the normal subject. The normal person grasps a specific identity in the conceptual structures; for him, the living processes of thought are symmetrical and match. This is how the normal subject “catches” the essence of the analogy and one can always wonder if a subject does not remain capable of understanding even when this comprehension is not adequately expressed through the formulation and the explicit account of it that he offers.⁷⁰

Living thought, then, does not consist in the act of subsuming under a category. The category imposes a signification upon the terms that it brings together, a signification that is external to the terms. By drawing from constituted language and the meaningful relations that it includes, Schneider succeeds in linking the eye to the ear as “sense organs.” In normal thought, the eye and the ear are immediately grasped according to the analogy of their function, and their relationship can only be fixed in a “common characteristic” and recorded in language because it has first been perceived in the nascent state in the singularity of vision and hearing.

Perhaps the objection will be raised that our critique only bears upon a rudimentary intellectualism that would absorb thought into a merely logical activity, whereas reflective analysis in fact goes back to the very foundations of predication, discovers a relational judgment behind the judgment of inherence, and finds behind subsumption (as a mechanical and formal operation) the categorial act by which thought invests the subject with the sense that is expressed in the predicate. Thus, our critique of the categorial function would have no other result than of awakening, behind the empirical use of the category, a transcendental use without which the first would indeed be incomprehensible. And yet, this distinction between the empirical use and the transcendental use conceals the difficulty rather than solving it. Critical philosophy doubles the empirical operations of thought with a transcendental activity charged with producing all of the syntheses of which empirical thought merely provides the pieces. But when I think of something now, the guarantee of a non-temporal synthesis is neither sufficient nor even necessary to ground my thought. It is now, in the living present, that the synthesis must be produced, otherwise thought would be cut off from its transcendental premises. Thus, it cannot be said that when I think I place myself back into the eternal subject I never ceased being, for the genuine subject of thought is the one who produces the present conversion and resumption, and it is he who communicates his life to the non-temporal phantom. We must, then, attempt to understand how temporal thought is secured upon itself and how it produces its own synthesis. If the normal subject immediately understands that the relation between the eye and vision is the same as the relation between the ear and hearing, this is because the eye and the ear are given to him immediately as ways of reaching a single world, and because he possesses a pre-predicative evidentness of a unique world, such that the equivalence of the “sense organs” and their analogy is read upon the things and can be lived prior to being conceived. The

Kantian subject posits a world, but, in order to be able to affirm a truth, the actual subject must first have a world or be in the world, that is, he must hold a system of significations around himself whose correspondences, relations, and participations do not need to be made explicit in order to be utilized.

When I move about in my house, I know immediately and without any intervening discourse that to walk toward the bathroom involves passing close to the bedroom, or that to look out the window involves having the fireplace to my left. In this small world, each gesture or each perception is immediately situated in relation to a thousand virtual coordinates. When I chat with a close friend, each of his words and each of mine contain, beyond what they signify for everyone else, a multitude of references to the principal dimensions of his personality and of mine, without our needing to evoke our previous conversations. These acquired worlds, which give my experience its secondary sense, are themselves cut out of a primordial world that grounds the primary sense of my experience. Similarly, there is a “world of thoughts,” a sedimentation of our mental operations, which allows us to count on our acquired concepts and judgments, just as we count upon the things that are there and that are given as a whole, without our having to repeat their synthesis at each moment. This is how for us there can be a sort of mental panorama with its accentuated regions and its confused regions, a physiognomy of questions, and intellectual situations such as research, discovery, and certainty. But this word “sedimentation” must not trick us: this contracted knowledge is not an inert mass at the foundation of our consciousness. For me, my apartment is not a series of strongly connected images. It only remains around me as my familiar domain if I still hold “in my hands” or “in my legs” its principal distances and directions, and only if a multitude of intentional threads run out toward it from my body. Likewise, my acquired thoughts are not an absolute acquisition; they feed off my present thought at each moment; they offer me a sense, but this is a sense that I reflect back to them. In fact, the acquisition that is available to us expresses, at each moment, the energy of our present consciousness. Sometimes it becomes weaker, such as happens with fatigue, and then my “world” of thought becomes impoverished and is reduced even to one or two obsessive ideas; sometimes, on the contrary, I am directed toward all of my thoughts, and each word that is spoken in front of me thus engenders questions or ideas, regroups and reorganizes the mental panorama, and appears with a precise physiognomy. The acquired, then, is only truly acquired if it is taken up in a new movement of thought, and a thought is only situated if it itself assumes its situation. The essence of consciousness is to provide itself with one or many worlds, to make its own thoughts exist *in front of* itself like things, and sketching out these landscapes and abandoning them indivisibly demonstrates its vitality. The structure “world,” with its double moment of sedimentation and spontaneity, is at the center of consciousness, and it is through a certain leveling out of the “world” that we will be able to understand Schneider's intellectual, perceptual, and motor disorders simultaneously and without thereby reducing them to each other.

i. Existential analysis of “perceptual disorders” and “intellectual disorders.”

The classical analysis of perception⁷¹ separates within perception the sensible givens and the signification that they receive through an act of the understanding. From this point of view, perceptual disorders must be either sensory deficiencies or gnostic disorders. Schneider's case reveals, however, deficiencies that have to do with the junction of sensitivity and signification, and which reveal the

existential conditioning of both. If the patient is shown a fountain pen such that the clip is not visible, the phases of recognition are as follows: the patient says: “it's black, blue, and bright. There is a white patch, it is oblong. It has the form of a stick. It could be some kind of instrument. It shines. It reflects light. It might also be a colored piece of glass.” The pen is then brought closer to the patient and the clip is turned toward him. He continues: “it must be a pencil or a fountain pen. (He touches his vest pocket). This is where it goes, for writing something down.”⁷² Language clearly intervenes in each phase of the recognition by providing possible significations for what is actually seen, and the recognition clearly progresses by following the connections of language, from “oblong” to “the form of a stick,” from “stick” to “instrument,” then to “instrument for writing something down,” and finally to “fountain pen.” The sensory givens are limited to suggesting these significations in the manner that a fact suggests to the physicist an hypothesis. The patient, like the scientist, verifies mediately and makes his hypothesis more precise by cross-checking the facts. He proceeds blindly toward the hypothesis that coordinates them all. This process brings to light, by contrast, the spontaneous method of normal perception, that sort of life of significations that renders the concrete essence of the object immediately readable and that only allows its “sensory properties” to appear through it. For Schneider this familiarity, this communication with the object is interrupted. For the normal person, the object is “speaking” [*parlant*] and meaningful, the arrangement of colors immediately “means” something, whereas for the patient the signification must be brought in from elsewhere through a genuine act of interpretation.

– Reciprocally, for the normal person, the subject's intentions are immediately reflected in the perceptual field: they polarize it, put their stamp on it, or finally, effortlessly give birth there to a wave of significations. For the patient, the perceptual field has lost this plasticity. If he is asked to construct a square out of four triangles identical to one that he is shown, he responds that it is impossible and that one can only construct two squares out of four triangles. The doctor insists by showing him that a square has two diagonals and can always be divided into four triangles. The patient responds: “Yes, but that is because the parts necessarily fit together. When a square is divided into four, then if the parts are brought together appropriately, the result must be a square.”⁷³ So he knows what a square and a triangle are, and the relation between these two significations does not escape him – at least after the doctor's explanations – and he understands that every square can be divided into triangles; but he fails to conclude that every (right isosceles) triangle can serve to construct a square with four times the surface area, because the construction of the square demands that the given triangles be assembled differently and that the sensory givens become the illustration of an imaginary sense. All things considered, the world no longer suggests any significations to him and, reciprocally, the significations that he considers are no longer embodied in the given world.

We shall conclude, in short, that the world no longer has a *physiognomy* for him.⁷⁴ This is what helps us to understand the strangeness of his drawings. Schneider never draws *according to* the model (*nachzeichnen* [traces]); perception is never directly continued into movement. He palpates the object with his left hand, recognizes certain particularities (a corner, a right angle), articulates his discovery, and finally traces an object freehand corresponding to the verbal articulation.⁷⁵ The translation of the perceived into movement passes through the express significations of language, whereas the normal

subject penetrates the object through perception and assimilates its structure, the object directly regulates his movements through his body.⁷⁶ This dialogue between the subject and the object, where the subject takes up the sense scattered across the object and the object gathers together the subject's intentions, namely, physiognomic perception, arranges a world around the subject that speaks to him on the topic of himself and places his own thoughts in the world. If this function is compromised for Schneider, we can expect that, *a fortiori*, perception of human events and of others will present deficiencies, for they presuppose the same taking up of the exterior in the interior and of the interior by the exterior. And indeed, if the patient is told a story, rather than grasping it as a melodic whole with its strong and weak beats and with its characteristic rhythm or flow, he only retains it as a series of facts that must be recorded one by one. This is why he only understands the story if pauses are included in the narration and are used to summarize briefly the essential aspects of what has just been recounted. When he retells the story in turn, it is never *according to* the narration that had just been given (*Nacherzählen* [a retelling]): he accentuates nothing, he only understands the progress of the story to the extent that he recounts it, and the narration is seemingly reconstituted piece by piece.⁷⁷ For the normal subject, then, the story has an essence that appears as the narration advances, without any explicit analysis, and that subsequently guides the reproduction of the narration. The story is, for him, a certain human event, recognizable by its style, and the normal subject “understands” here because he has the power of living – beyond his immediate experience – the events indicated by the narration. In general, nothing is present for the patient other than what is given immediately. Another's thought, since he never experiences it directly, will never be present to him.⁷⁸ Another's words are for him signs that he must decode one by one, whereas for the normal subject these words are the transparent envelope of a sense *in* which he could live. For Schneider, words, like events, are not the motive of a taking up or of a projection, but merely the opportunity for a methodical interpretation. Another person, like an object, “says” nothing to him and, although the phantoms that appear to him are certainly not devoid of the intellectual signification that is obtained by analysis, they are devoid of the primordial signification obtained through coexistence.

Disorders that are properly intellectual – those of judgment and of signification – will not be able to be considered as ultimate deficiencies, and will now have to be put back into the same existential context. Consider “number blindness,”⁷⁹ for example. It was possible to show that the patient – who is capable of counting, adding, subtracting, multiplying, or dividing with regard to objects placed in front of him – cannot, however, imagine the number; all of these results are obtained through ritual procedures with which he has no meaningful relation. He knows the series of numbers by heart and recites it mentally while noting on his fingers the objects to be counted, added, subtracted, or divided: “the number has but a membership in the series of numbers, it has no signification as a fixed size, as a group, or as a determinate measure.”⁸⁰ Of two numbers, the greater is the one that comes “after” in the series of numbers. When he is asked to complete the equation “5 plus 4 minus 4,” he executes the operation in two steps without “noticing anything peculiar.” He simply agrees, if it is pointed out to him, that the number 5 “remains.” He does not understand that “doubling half” of a number is this very same number.

Should we thus conclude that he has lost “number” as a category or as a schema? But when he glances over the objects to be counted while “noting” each of them on his fingers, even if he often confuses objects already counted with those not yet counted, even if the synthesis is confused, he still clearly possesses the notion of a synthetic operation that is precisely what we call “numeration.” And reciprocally, for the normal subject the series of numbers as a kinetic melody, more or less devoid of authentically numerical sense, is most often substituted for the concept of number. Number is never a pure concept whose absence would allow us to define Schneider's mental state, it is a structure of consciousness that includes the more and the less. The genuine act of counting requires of the subject that his operations, as they unfold and cease to occupy the center of his consciousness, not cease to be there for him and rather constitute a *ground* upon which subsequent operations are established. Consciousness trails behind itself the completed syntheses, they remain available, they could be reactivated, and as such they are taken up and transcended in the total act of numeration. What is called pure number or authentic number is only a promotion or an extension through repetition of the constitutive movement of every perception. For Schneider, the conception of number is affected only insofar as it presupposes eminently the power of deploying a past in order to go toward a future. This existential basis of intelligence is affected much more than intelligence itself, for, as has been shown,⁸² Schneider's general intelligence is intact: his responses may be slow, but they are never meaningless, they are those of a mature and reflective man who is interested in the doctor's experiments.

Beneath intelligence, understood as an anonymous function or as a categorial operation, we must acknowledge a personal core that is the patient's being and his power of existing. Here is where the disorder resides. Schneider would still like to form political or religious opinions, but he knows that it is useless to try. “He must now be content with general beliefs and without being able to express them.”⁸³ He never sings nor whistles on his own.⁸⁴ We will see below that he never takes the initiative sexually. He never goes out for a walk, but always to run an errand, and he does not recognize Professor Goldstein's house when walking by it “because he has not gone out with the intention of going there.”⁸⁵ Just as he needs to gain a “hold” on his own body through preparatory movement prior to executing movements that are not traced out in advance in a familiar situation, so too a conversation with another person fails to constitute for him a situation that is meaningful in itself and that might solicit impromptu responses. He can only speak according to a plan settled in advance: “he cannot give himself over to the inspiration of the moment in order to find the necessary thoughts in response to a complex situation in the conversation, and this is the case whether it is a question of new points of view or of old ones.”⁸⁶ There is something meticulous and serious in all of his behavior, which comes from the fact that he is incapable of playing. To play is to place oneself momentarily in an imaginary situation, to amuse oneself in changing one's “milieu.” The patient, however, cannot enter into a fictional situation without converting it into a real situation: he does not distinguish between a riddle and a problem.⁸⁷ “For Schneider, the possible situation is at each moment so narrow that two regions of the milieu, if they do not have something in common for him, cannot simultaneously become a situation.”⁸⁸ If someone is speaking to him, he does not hear the noise of another conversation in the next room; if someone brings a dish to the table, he never wonders where the dish came from. He declares that one sees only in the

direction that one looks, and only the objects upon which one focuses.⁸⁹ The future and the past are for him nothing but the “shriveled up” continuations of the present. He has lost “our power of seeing according to the temporal vector.”⁹⁰ He cannot survey his past from above and unhesitatingly meet up with it by going from the whole to the parts. Rather, he reconstitutes it by beginning with a fragment that has maintained its sense and that serves him as a “fulcrum.”⁹¹ When he complains about the weather, he is asked if he feels better during the winter. He responds: “I can't say now ... for the moment, I can't say anything.”⁹²

j. The “intentional arc.”*

Thus, all of Schneider's disorders can be reduced to a unity, but this is not the abstract unity of the “representation function.” Schneider is “bound” to the actual, and he “lacks freedom,”⁹³ he lacks the concrete freedom that consists in the general power of placing oneself in a situation. We discover beneath intelligence and beneath perception a more fundamental function: “a vector moving in every direction, like a searchlight, by which we can orient ourselves toward anything, in ourselves or outside of ourselves, and by which we can have a behavior with regard to this object.”⁹⁴ But again, the comparison to a searchlight is not a good one, since it takes for granted the given objects upon which intelligence projects its light, whereas the core function we are speaking of here – prior to making us see or know objects – first more secretly brings them into existence for us. So let us say instead, by borrowing a term from another work,⁹⁵ that the life of consciousness – epistemic life, the life of desire, or perceptual life – is underpinned by an “intentional arc” that projects around us our past, our future, our human milieu, our physical situation, our ideological situation, and our moral situation, or rather, that ensures that we are situated within all of these relationships. This intentional arc creates the unity of the senses, the unity of the senses with intelligence, and the unity of sensitivity and motricity. And this is what “goes limp” in the disorder.

The study of a pathological case has thus allowed us to catch sight of a new mode of analysis – existential analysis – that goes beyond the classical alternatives between empiricism and intellectualism, or between explanation and reflection. If consciousness were a sum of psychic facts, then each disturbance should be elective. If consciousness were a “representation function” or a pure power of signifying, then it could exist or not exist (and everything else along with it), but it could not cease to exist after having existed, nor could it become ill, that is, it could not be altered. Finally, if consciousness is an activity of projection, which deposits objects around itself like traces of its own acts, but which relies upon them in order to move on to new acts of spontaneity, then we understand simultaneously that every deficiency of “contents” has an effect upon the whole of experience and begins its disintegration, that every pathological weakening has to do with all of consciousness – and that, nevertheless, the disorder each time attacks consciousness from a certain “side,” that in each case certain symptoms are predominant in the clinical picture of the illness, and finally that consciousness is vulnerable and that consciousness itself can suffer the illness. By attacking the “visual sphere,” the illness is not limited to destroying certain conscious contents, namely, “visual representations” or vision in the literal sense; rather, it attacks vision in a figurative sense, of which the former is but the model or

the emblem – the power of “surveying” or “dominating” (*überschauen*)⁹⁶ simultaneous multiplicities and a certain manner of positing the object or of being conscious. But since this type of consciousness is nevertheless merely the sublimation of sensory vision, and since it is schematized at each moment in the dimensions of the visual field (admittedly by investing them with a new sense), we can understand that this general function has psychological roots. Consciousness freely develops the visual givens beyond their own sense, it makes use of them in order to express its acts of spontaneity, as can be seen clearly in the semantic evolution that invests the terms “intuition,” “evidentness,” and the “natural light” with an ever-richer sense. But, reciprocally, not one of these terms can be understood, in the final sense that history has given them, without reference to the structures of visual perception. One cannot say that man sees because he is Spirit, nor that he is Spirit because he sees: “to see as a man sees” and “to be Spirit” are synonymous. To the extent that consciousness is only consciousness of something by allowing its wake to trail behind itself, and to the extent that, to think an object, consciousness must rely upon a previously constructed “world of thought,” there is always a depersonalization at the heart of consciousness. From this appears the principle of a foreign intervention: consciousness can be ill, the world of its thoughts can fall to pieces; or rather, since the “contents” dissociated by the illness did not figure in normal consciousness as parts and only served as the supports for significations that transcended them, consciousness can be seen attempting to maintain its superstructures even though their foundation has collapsed. It mimics its customary operations, but without the power of obtaining their intuitive realization and without the power of hiding the strange deficiency that steals from them their full sense. If the mental illness is, in turn, tied to a bodily accident, then this is understood, in principle, in the same way. Consciousness projects itself into a physical world and has a body, just as it projects itself into a cultural world and has a habitus. This is because it can only be consciousness by playing upon significations given in the absolute past of nature or in its personal past, and because every lived form tends toward a certain generality, whether it be the generality of our habitus or rather that of our “bodily functions.”

k. The intentionality of the body.

Finally, these clarifications allow us to understand motricity unequivocally as original intentionality. Consciousness is originarily not an “I think that,” but rather an “I can.”⁹⁷ Schneider's motor disorder cannot, any more than his visual disorder, be reduced to a weakness in the general function of representation. Vision and movement are specific ways of relating to objects and, if a single function is expressed throughout all of these experiences, then it is the movement of existence, which does not suppress the radical diversity of contents, for it does not unite them by placing them all under the domination of an “I think,” but rather by orienting them toward the inter-sensory unity of a “world.” Movement is not a movement in thought, and bodily space is not a space that is conceived or represented.

Each voluntary movement takes place in a milieu, against a background determined by the movement itself (...). We execute our movements in a space that is not “empty” and without relation to them, but which is, on the contrary, in a highly determined relation with them: movement and background are only, in fact, moments artificially separated from a single whole.⁹⁸

The gesture of reaching one's hand out toward an object contains a reference to the object, not as a representation, but as this highly determinate thing toward which we are thrown, next to which we are through anticipation, and which we haunt.⁹⁹ Consciousness is being toward the thing through the intermediary of the body. A movement is learned when the body has understood it, that is, when it has incorporated it into its "world," and to move one's body is to aim at the things through it, or to allow one's body to respond to their solicitation, which is exerted upon the body without any representation. Motricity is thus not, as it were, a servant of consciousness, transporting the body to the point of space that we imagine beforehand. For us to be able to move our body toward an object, the object must first exist for it, and hence our body must not belong to the region of the "in-itself." Objects no longer exist for the arm of the person suffering from apraxia, and this is what renders his arm immobile. Cases of pure apraxia, where the perception of space is intact, where even the "intellectual notion of the gesture to be performed" does not seem confused, and where nevertheless the patient does not know how to reproduce a triangle,¹⁰⁰ or cases of constructive apraxia, where the subject exhibits no gnosic disorder, except that which has to do with the localization of stimuli upon the body, and yet is not capable of reproducing a cross, a v, or an o¹⁰¹ – all of these cases show clearly that the body has its world and that objects or space can be present to our knowledge without being present to our body.

1. The body is not in space, it inhabits space.

Thus, we must not say that our body is *in* space, nor for that matter *in* time. It *inhabits* space and time. If my hand executes a complicated movement in the air, I do not have to add together all the movements in one direction and subtract the movements in the other in order to know its final position. "Every recognizable change enters into consciousness already charged with its relations to something that has gone before, just as on a taximeter the distance is presented to us as already transformed into shillings and pence."¹⁰² At each moment, previous postures and movements constantly provide a standard of measure. This has nothing to do with the visual or motor "memory" of the hand's starting point: cerebral lesions can leave the visual memory intact while suppressing the consciousness of movement and, as for "motor memory," it clearly could not determine the present position of the hand if the perception of where it was born had not itself included an absolute consciousness of the "here," without which one would be sent from memory to memory and would never have a present perception. Just as it is necessarily "here," the body necessarily exists "now"; it can never become "past." Even if we cannot preserve the living memory of the illness when we are healthy, nor the living memory of our body as a child when we have become an adult, these "gaps in memory" do nothing but express the temporal structure of our body. At each moment in a movement, the preceding instant is not forgotten, but rather is somehow fit into the present, and, in short, the present perception consists in taking up the series of previous positions that envelop each other by relying upon the current position. But the imminent position is itself enveloped in the present, and through it so too are all of those positions that will occur throughout the movement. Each moment of the movement embraces its entire expanse and, in particular, its first moment or kinetic initiation inaugurates the link between a here and a there, between a now and a future that the other moments will be limited to developing.

Insofar as I have a body and insofar as I act in the world through it, space and time are not for me a mere summation of juxtaposed points, and no more are they, for that matter, an infinity of relations synthesized by my consciousness in which my body would be implicated. I am not in space and in time, nor do I think space and time; rather, I am of space and of time;¹⁰³ my body fits itself to them and embraces them. The scope of this hold measures the scope of my existence; however, it can never in any case be total. The space and time that I inhabit are always surrounded by indeterminate horizons that contain other points of view. The synthesis of time, like that of space, is always to be started over again. The motor experience of our body is not a particular case of knowledge; rather, it offers us a manner of reaching the world and the object, a “praktognosia,”¹⁰⁴ that must be recognized as original, and perhaps as originary. My body has its world, or understands its world without having to go through “representations,” or without being subordinated to a “symbolic” or “objectifying function.” Certain patients, if they stand next to the doctor and observe his movements in a mirror, can imitate the doctor's movements and raise their right hand to their right ear and their left hand to their nose. But they cannot do so if they are in front of him. Head explained the patient's failure through the inadequacy of his “formulation”: the imitation of the gesture would be mediated through a verbal translation. In fact, the formulation can be precise without the imitation being successful, and the imitation can be successful without any formulation at all. Certain authors in this field¹⁰⁵ thus introduce, if not a verbal symbolism, then at least a general symbolic function, a capacity for “transposing” of which imitation would be, like perception or objective thought, merely a particular case. But it is clear that this general function does not explain adapted action. For these patients are capable not merely of formulating the movement to be accomplished, but moreover of representing it to themselves. They know very well what they have to do, and nevertheless, rather than bringing their right hand to their right ear and their left hand to their nose, they touch one ear with each hand, or even their nose and one eye, or one ear and one eye.¹⁰⁶ What has become impossible is the application and adjustment of the objective definition to their own body. In other words, right hand and left hand, eye and ear are still given to them as absolute locations, but are no longer inserted in a system of correspondence that links them to the homologous parts of the doctor's body and that makes them available for imitation, even when the doctor is facing the patient. To be able to imitate the gestures of someone facing me, I need not know explicitly that “the hand appearing to the right of my visual field is my partner's left hand.” The patient is precisely the one who resorts to such explanations. In normal imitation, the subject's left hand is immediately identified with his partner's, the subject's action immediately adheres to his model, the subject projects himself into or “irrealizes” himself in the model,¹⁰⁷ identifies himself with the model, and the change of coordinates is eminently contained in this existential operation. This is because the normal subject has his body not only as a system of current positions, but also, and consequently, as an open system of an infinity of equivalent positions in different orientations. What we called the “body schema” is precisely this system of equivalences, this immediately given invariant by which different motor tasks are instantly transposable. This is to say that the body schema is not merely an experience of my body, but rather an experience of my body in the world, and that it gives a motor sense to the verbal instructions. The function destroyed in the disorders of apraxia is thus surely a motor function.

In cases of this genre, it is not the symbolic or significative function in general that is affected, but rather a much more originary function, one that has a motor characteristic, namely, the capacity for motor differentiation of the dynamic body schema.¹⁰⁸

The space through which normal imitation moves is not (in contrast to concrete space with its absolute locations) an “objective space” or a “space of representation” founded upon an act of thought. It is already sketched out in the structure of my body, it is my body's inseparable correlate. “Taken in its pure state, motricity already possesses the elementary power of sense-giving (*Sinngebung*).”¹⁰⁹ Even if, in what follows, thought and the perception of space are liberated from motricity and from being in space, in order for us to be able to imagine space, it must first be introduced into it through our body, which must have given us the first model of transpositions, equivalences, and identifications that turns space into an objective system and allows our experience to be an experience of objects and to open onto an “in-itself.” “Motricity is the primary sphere in which the sense of all significations (*der Sinn aller Signifikationen*) is first given in the domain of represented space.”¹¹⁰

m. Habit as the motor acquisition of a new signification.

Acquiring a habit as the reworking and renewal of the body schema presents significant difficulties for classical philosophies, which are always inclined to conceive of synthesis as intellectual synthesis. It is true, of course, that what links elementary movements, reactions, and “stimuli” together in habit is not an external association.¹¹¹ Every mechanistic theory runs into the fact that the learning process is systematic: the subject does not weld individual movements to individual stimuli, but rather acquires the power of responding with a certain type of solution to a certain form of situation. The situations may differ widely from case to case, the responding movements may be entrusted sometimes to one effector organ and sometimes to another, and situations and responses resemble each other in the different cases much less through the partial identity of elements than by the community of their sense. Must we thus place an act of the understanding at the origin of the habit that would first organize the habit's elements only to later withdraw from it?¹¹² For example, in learning the habit of a certain dance, do we not find the formula of the movement through analysis and then recompose it, taking this ideal sketch as a guide and drawing upon already acquired movements (such as walking and running)? But in order for the new dance to integrate particular elements of general motricity, it must first have received, so to speak, a motor consecration. The body, as has often been said, “catches” (*kapiert*) and “understands” the movement. The acquisition of the habit is surely the grasping of a signification, but it is specifically the motor grasping of a motor signification. But what exactly does this mean?

Without any explicit calculation, a woman maintains a safe distance between the feather in her hat and objects that might damage it; she senses where the feather is, just as we sense where our hand is.¹¹³ If I possess the habit of driving a car, then I enter into a lane and see that “I can pass” without comparing the width of the lane to that of the fender, just as I go through a door without comparing the width of the door to that of my body.¹¹⁴ The hat and the automobile have ceased to be objects whose size and volume would be determined through a comparison with other objects. They have become voluminous powers

and the necessity of a certain free space. Correlatively, the subway door and the road have become restrictive powers and immediately appear as passable or impassable for my body and its appendages. The blind man's cane has ceased to be an object for him, it is no longer perceived for itself; rather, the cane's furthest point is transformed into a sensitive zone, it increases the scope and the radius of the act of touching and has become analogous to a gaze. In the exploration of objects, the length of the cane does not explicitly intervene nor act as a middle term: the blind man knows its length by the position of the objects, rather than the position of the objects through the cane's length. The position of objects is given immediately by the scope of the gesture that reaches them and in which, beyond the potential extension of the arm, the radius of action of the cane is included. If I want to become habituated to a cane, I try it out, I touch some objects and, after some time, I have it "in hand": I see which objects are "within reach" or out of reach of my cane. This has nothing to do with a quick estimate or a comparison between the objective length of the cane and the objective distance of the goal to be reached. Places in space are not defined as objective positions in relation to the objective position of our body, but rather they inscribe around us the variable reach of our intentions and our gestures. To habituate oneself to a hat, an automobile, or a cane is to take up residence in them, or inversely, to make them participate within the voluminosity of one's own body. Habit expresses the power we have of dilating our being in the world, or of altering our existence through incorporating new instruments.¹¹⁵ One can know how to type without knowing how to indicate where on the keyboard the letters that compose the words are located. Knowing how to type, then, is not the same as knowing the location of each letter on the keyboard, nor even having acquired a conditioned reflex for each letter that is triggered upon seeing it.

But if habit is neither a form of knowledge nor an automatic reflex, then what is it? It is a question of a knowledge in our hands, which is only given through a bodily effort and cannot be translated by an objective designation. The subject knows where the letters are on the keyboard just as we know where one of our limbs is – a knowledge of familiarity that does not provide us with a position in objective space. The movement of his fingers is not presented to the typist as a spatial trajectory that can be described, but merely as a certain modulation of motricity, distinguished from every other through its physiognomy. The question is often presented as if the perception of the letter written on the paper came to awaken the representation of the same letter, which in turn evoked the representation of the movement necessary to reach it on the keyboard. But this language is mythological. When I glance over the text offered to me, there are no perceptions awakening representations, but rather wholes that arrange themselves at the present moment, endowed with a typical or familiar physiognomy. When I take my place before my machine, a motor space stretches beneath my hands where I will play out what I have read. The word that is read is a modulation of visual space, the motor execution is a modulation of manual space, and the whole question is how a certain physiognomy of "visual" wholes can call forth a certain style of motor responses, how each "visual" structure in the end provides its own motor essence, without our having to spell out the word or to spell out the movement in order to translate the word into movement. But this power of habit is not distinguished from the one we have over our body in general. If I am told to touch my ear or my knee, I bring my hand to my ear or to my knee by the shortest path without my having to imagine the position of my hand at the outset, the position of my ear, or the trajectory from one to the other. We said above that in the acquisition of habit it is the body that "understands." This formula will seem absurd if "understanding" is the act of subsuming a sensory given

under an idea, and if the body is a mere object. But the phenomenon of habit in fact leads us to rework our notion of “understanding” and our notion of the body. To understand is to experience [*éprouver*] the accord between what we aim at and what is given, between the intention and the realization – and the body is our anchorage in a world. When I bring my hand toward my knee, I experience at each moment of the movement the realization of an intention that did not aim at my knee as an idea, or even as an object, but rather as a present and real part of my living body, and ultimately as a point of passage in my perpetual movement toward a world. When the typist executes the necessary movements on the keyboard, these movements are guided by an intention, but this intention does not posit the keys as objective locations. The subject who learns to type literally incorporates the space of the keyboard into his bodily space.

The example of instrumentalists demonstrates even more clearly how habit resides neither in thought nor in the objective body, but rather in the body as the mediator of a world. It is said that an experienced organist¹¹⁶ is capable of playing an organ with which he is unfamiliar and that has additional or fewer keyboards, and whose stops are differently arranged than the stops on his customary instrument. He needs but an hour of practice to be ready to execute his program. Such a brief apprenticeship prohibits the assumption that new conditioned reflexes are simply substituted for the already established collection, unless, that is, they together form a system and if the change is global, but this would be to go beyond the mechanistic theory since in that case the reactions would be mediated by a total hold on the instrument. Shall we say, then, that the organist analyzes the organ, that he forms and maintains a representation of the stops, pedals, and keyboards, as well as their relation in space? But during the short rehearsal that precedes the concert he hardly behaves like someone who wants to draw up a plan. He sits on the bench, engages the pedals, and pulls out the stops, he sizes up the instrument with his body, he incorporates its directions and dimensions, and he settles into the organ as one settles into a house. He does not learn positions in objective space for each stop and each pedal, nor does he entrust such positions to “memory.” During the rehearsal – just as during the performance – the stops, the pedals, and the keyboards are only presented to him as powers of such and such an emotional or musical value, and their position as those places through which this value appears in the world. Between the musical essence of the piece such as it is indicated in the score and the music that actually resonates around the organ, such a direct relationship is established that the body of the organist and the instrument are nothing other than the place of passage of this relation. From then on, the music exists for itself, and everything else exists through it.¹¹⁷ There is no place here for a “memory” of the location of the stops, and the organist does not play within objective space. In fact, his rehearsal gestures are gestures of consecration: they put forth affective vectors, they discover emotional sources, and they create an expressive space, just as the gestures of the augur define the *templum*.

The entire problem of habit here is to determine how the musical signification of the gesture can be condensed into a certain locality to the extent that, by entirely giving himself over to the music, the organist reaches for precisely the stops and the pedals that will actualize it. Of course, the body is eminently an expressive space. No sooner have I formed the desire to take hold of an object than already, at a point in space that I was not thinking about, my hand as that power for grasping rises up toward the object. I do not move my legs insofar as they are in space and eighty centimeters from my

head, but rather insofar as their ambulatory power continues my motor intention downward. The principal regions of my body are consecrated to actions, the parts of my body participate in their value, and the question as to why common sense places the seat of thought in the head is the same as the question of how the organist distributes musical significations in the space of the organ. But our body is not merely one expressive space among all others, for that would be merely the constituted body. Our body, rather, is the origin of all the others, it is the very movement of expression, it projects significations on the outside by giving them a place and sees to it that they begin to exist as things, beneath our hands and before our eyes. Even if our body does not impose definite instincts upon us from birth, as the animal's body does, then it at least gives the form of generality to our life and prolongs our personal acts into stable dispositions. Our nature, in this sense, is not an ancient custom, since custom presupposes nature's form of passivity. The body is our general means of having a world. Sometimes it restricts itself to gestures necessary for the conservation of life, and correlatively it posits a biological world around us. Sometimes, playing upon these first gestures and passing from their literal to their figurative sense, it brings forth a new core of signification through them – this is the case of new motor habits, such as dance. And finally, sometimes the signification aimed at cannot be reached by the natural means of the body. We must, then, construct an instrument, and the body projects a cultural world around itself. At all levels, the body exercises the same function, which is to lend “a bit of renewable action and independent existence”¹¹⁸ to the momentary movements of freedom. Habit is but a mode of this fundamental power. The body, then, has understood and the habit has been acquired when the body allows itself to be penetrated by a new signification, when it has assimilated a new meaningful core.

What we have discovered through the study of motricity is, in short, a new sense of the word “sense.” The strength of intellectualist psychology, as well as of idealist philosophy, comes from the ease with which they show that perception and thought have an intrinsic sense and cannot be explained through an external association of fortuitously assembled contents. The *Cogito* was the moment of insight into this interiority. And yet, every signification was simultaneously conceived as an act of thought, as the operation of a pure “I”; if intellectualism easily won out over empiricism, it itself remained incapable of accounting for the variety of our experience, for the regions of non-sense in our experience, and for the contingency of its contents. The experience of the body leads us to recognize an imposition of sense that does not come from a universal constituting consciousness, a sense that adheres to certain contents. My body is this meaningful core that behaves as a general function and that nevertheless exists and that is susceptible to illness. In the body we learn to recognize this knotting together of essence and existence that we will again meet up with in perception more generally, and that we will then have to describe more fully.